

SNAKE BITE

1. Case definition

* **Is it a snakebite?**

Ask carefully about the circumstances: was the snake seen? Is it a scorpion, spider or insect bite? Which snake is it? Is there envenomation?

- local signs: oedema, pain, adenopathy
- general signs: lipothymia, temperature, nausea, vomiting

* **Are there any signs of complications?**

- Local: sero-sanguinous bullae, necrosis, superinfection.
- General :
 - a) Hemorrhagic syndrome: dreaded, can occur several days after the bite. Watch epistaxis, hematemesis, hematuria, blood loss through the anus, signs of internal bleeding with rapid pulse, abdominal pain, paleness, low blood pressure, etc. The patient must be systematically questioned and informed of this danger, which makes evacuation to the district hospital imperative.
 - b) Anaphylactic shock: occurs within minutes or hours of envenomation. Check A.T., heart rate, consciousness.

2. Case management

What not to do:

- incision (black stone), local infiltration, tourniquet, local application of ice.

In all cases of envenomation:

- Reassuring and informing the patient
- Keep the patient under observation for 5 , in bed with the bitten limb slightly elevated if there is any sign of envenomation.
- Watch for complications.
- Strict disinfection, VAT, SAT if incised for black stone or traditional treatment.
- Paracetamol 500mg 2x 2cp/d p. 5 days.

In cases with complications: REFER urgently

Evacuate to the district hospital and take the first steps:

In case of local complications: **Procaine benzyl penicillin 2.4 MU IM** and **Metronidazole 2cp** - *treatment to be continued at the referral center according progress.*

In case of bleeding syndrome: *avoid ASA, other anti-inflammatories and IM injections.* In case of shock only: **Dexamethasone 1 to 2 ampoules, promethazine, Ringer's infusion** - see also **FT hypovolemic shock**.

In case eye damage by spitting naja :

Rinse the eye thoroughly with pure water, apply 1% tetracycline ointment and apply an occlusive bandage for 48 hours (*this venom causes corneal erosions that can become superinfected and lead to secondary blindness*).